PACKAGE INSERT

PRODUCT : TRAMADA CAPSULES 50 mg
REF.NO.: BPFK/20/01030103A

REG.NO.: MAL20031921AZ

NAME AND DOSAGE FORM OF PRODUCT
Tramada capsules 50 mg

THERAPEUTIC CLASS : DESCRIPTION :

TRAMADA, in the form of white powder, is filled in a hard gelatin capsule size 4, Opaque pink colour, imprinted with black-figure C.B.LAB on body and cap.

NAME (S) AND STRENGTH (S) OF ACTIVE INGREDIENT (S) :

Each Tramada Capsule contains Tramadol Hydrochloride 50 mg.

ACTION AND MODE OF MECHANISMS OF ACTION :

TRAMADA is a centrally active opioid analgesic. Its activities are attributed to the stimulation of opioid receptors.

PHARMACOLOGY (SUMMARY OF PHARMACODYNAMICS AND PHARMACOKINETICS) :

TRAMADA acts as an opiate agonist, apparently by selective activity at the u-receptor. In addition to opiate agonist activity, tramadol inhibits reuptake of certain monoamines (norepinephrine, serotonin, which appears to contribute to the drug's analgesic effect).

The onset and peak of analgesic occurs within 1 and 2-4 hours, respectively, after oral administration of tramadol hydrochloride, peak plasma concentrations are achieved about 2 hours after oral administration, corresponding to the time of peak analgesic effect.

The duration of analgesia produced by a single oral dose tramadol hydrochloride has been reported to be about 3-6 hours.

TOXICOLOGY :

TRAMADA shares the toxicologic effects of opiate agonists, the respiratory depressant effects of the drug are less than those of morphine, and usually are not clinically important at usual oral doses.

INDICATIONS :

Treatment of moderate to severe pain.

CONTRAINDICATIONS

* TRAMADA must not be administered in known hypersensitivity towards tramadol or any of the excipients or in acute intoxication with alcohol, hypnotics, analgesics, or other CNS-acting drugs.

* TRAMADA must not be used for narcotic withdrawal treatment.

* To be used with special care in opioid-dependence, reduced level of consciousness of unclear origin, disorders of the respiratory centre of function and in increased intracranial pressure.

* Must not be used with MAO inhibitors or within 14 days of their withdrawal.

* Children younger than 18 years to treat pain after surgery to remove the tonsils and/or adenoids.

* Adolescents between 12 and 18 years who are obese or have conditions such as obstructive sleep apnea or severe lung disease, which may increase the risk of serious breathing problems.

SIDE EFFECT / ADVERSE REACTIONS.

* Common side-effects are nausea, vomiting, dizziness, sweating, dry mouth and fatigue.

* Cardiovascular regulation (palpitation, tachycardia postural hypotension up to cardiovascular collapse).

* Headache, retching, constipation, gastro-intestinal irritation and dermal reactions.

* Very rare cases of motorial weakness, changes in appetite, micturition disorders and psychic side-effects which include changes in mood, changes in activity and changes in cognitive and sensorial capacity.

* In isolated cases, cerebral convulsions, allergic reactions and shock has been reported.

* Respiratory depression (rare).

Postmarketing Experience:
Serotonin syndrome (See Precautions/Warnings)
Adrenal insufficiency (See Precautions/Warnings)

Androgen deficiency: Cases of androgen deficiency have occurred with chronic use of opioids. Chronic use of opioids may influence the hypothalamic-pituitary-gonadal axis, leading to androgen deficiency that may manifest as low libido, impotence, erectile dysfunction, amenorrhea, or infertility.
The causal role of opioids in the clinical syndrome of hypogonadism is unknown because the various medical, physical, lifestyle, and psychological stressors that may influence gonadal hormone levels have not been adequately controlled for in studies conducted to date. Patients presenting with symptoms of androgen deficiency should undergo laboratory evaluation.
Infertility: Chronic use of opioids may cause reduced fertility in females and males of reproductive potential. It is not known whether these effects on fertility are reversible.

PRECAUTIONS / WARNINGS :

Use with care in patients with increased reactivity to opioids. Patients known to suffer from convulsions should be carefully monitored. TRAMADA has a dependence potential. On long-term use, tolerance, psychic and physical dependence may develop. Therefore, in patients with a tendency towards drug abuse of dependence, treatment should only be given for short periods under strict medical supervision.

Effects on the ability to drive or operate machinery: Even on recommended dosage, TRAMADA may impair patient's capacity to drive or operate machines especially in conjunction with alcohol.

Paediatric population
The safety and efficacy of TRAMADA has not been studied in the paediatric population. Therefore, use of TRAMADA is not recommended in patients under 12 years of age.

Respiration depression
Administer TRAMADA cautiously in patients at risk for respiratory depression, including patients with substantially decreased respiratory reserve, hypoxia, hypercapnia, or pre-existing respiratory depression, as in these patients, even therapeutic doses of TRAMADA may decrease respiratory drive to the point of apnea. In these patients, alternative non-opioid analgesics should be considered. When large doses of tramadol are administered with anaesthetic medications or alcohol, respiratory depression may result. Respiratory depression should be treated as an overdose. If naloxone is to be administered, use cautiously because it may precipitate seizures.

Cytochromes P450 (CYP) 2D6 Ultra-Rapid Metabolism
Some individuals may be CYP2D6 ultra-rapid metabolisers. These individuals convert tramadol more rapidly than other people into its more potent opioid metabolites O-desmethyltramadol (M1). This rapid conversion could result in higher-than-expected opioid-like side effects including life-threatening respiratory depression. The prevalence of this CYP2D6 phenotype varies widely and has been estimated at 0.5 to 1% in Chinese, Japanese and Hispanics, 1 to 10% in Caucasians, 3% in African Americans, and 16-28% in North Africans, Ethiopians, and Arabs. Data are not available for other ethnic groups.

Risks from Concomitant Use with Benzodiazepines
Profound sedation, respiratory depression, coma, and death may result from the concomitant use of TRAMADA with benzodiazepines. Observational studies have demonstrated that concomitant use of opioids and benzodiazepines increases the risk of drug-related mortality compared to use of opioids alone. Because of these risks, reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate.
If the decision is made to newly prescribe a benzodiazepine and an opioid together, prescribe the lowest effective dosages and minimum durations of concomitant use.
If the decision is made to prescribe a benzodiazepine in a patient already receiving an opioid, prescribe a lower initial dose of the benzodiazepine than indicated in the absence of an opioid, and titrate based on clinical response.
If the decision is made to prescribe an opioid in a patient already taking a benzodiazepine, prescribe a lower initial dose of the opioid, and titrate based on clinical response.
Follow patients closely for signs and symptoms of respiratory depression and sedation. Advise both patients and caregivers about the risks of respiratory depression and sedation when TRAMADA is used with benzodiazepines.
Advise patients not to drive or operate heavy machinery until the effects of concomitant use of the benzodiazepine have been determined. Screen patients for risk of substance use disorders, including opioid abuse and misuse, and warn them of the risk for overdose and death associated with the use of benzodiazepines (See Drug Interactions).

Serotonin Syndrome with Concomitant Use of Serotonergic Drugs
Cases of serotonin syndrome, a potentially life-threatening condition, have been reported during concurrent use of tramadol with serotonergic drugs (See Drug Interactions). This may occur within the recommended dosage range.

Serotonin syndrome symptoms may include mental-status changes (e.g. agitation, hallucinations, coma), autonomic instability (e.g. tachycardia, labile blood pressure, hyperthermia), neuromuscular aberrations (e.g. hyperreflexia, incoordination) and/or gastrointestinal symptoms (e.g. nausea, vomiting, diarrhoea) and can be fatal (See Drug Interactions). The onset of symptoms generally occurs within several hours to a few days of concomitant use, but may occur later than that. Discontinue TRAMADA if serotonin syndrome is suspected.

Adrenal Insufficiency
Cases of adrenal insufficiency have been reported with opioid use, more often following greater than one month of use. Presentation of adrenal insufficiency may include non-specific symptoms and signs including nausea, vomiting, decreased appetite, fatigue, weakness, dizziness, and low blood pressure. If adrenal insufficiency is suspected, confirm the diagnosis with diagnostic testing as soon as possible. If adrenal insufficiency is diagnosed, treat with physiologic replacement dosing of corticosteroids. Wean the patient off of the opioid to allow adrenal function to recover and continue corticosteroid treatment until adrenal function recovers. Other opioids may be tried as some cases reported use of a different opioid without recurrence of adrenal insufficiency. The information available does not identify any particular opioids as being more likely to be associated with adrenal insufficiency.

Sexual Function/Reproduction
Long term use of opioids may be associated with decreased sex hormone levels and symptoms such as low libido, erectile dysfunction, or infertility (See Postmarketing Experience)

PREGNANCY AND LACTATION :

Pregnancy
Tramadol has been shown to cross the placenta. There are no adequate and well-controlled studies in pregnant women. Safe use in pregnancy has not been established. TRAMADA is not recommended for pregnant women.

Lactation
Approximately 0.1% of the maternal dose of tramadol is excreted in breast milk. In the immediate post-partum period, for maternal oral daily dosage up to 400 mg, this corresponds to a mean amount of tramadol ingested by breast-fed infants of 3% of the maternal weight-adjusted dosage. For this reason, tramadol should not be used during lactation or alternatively, breast-feeding should be discontinued during treatment with tramadol. Discontinuation of breast-feeding is generally not necessary following a single dose of tramadol.

DRUG INTERACTION :

On the concomitant administration with substances which also act on the central nervous system (e.g. tranquillizers, hypnotics) the central side-effects (sedation and drowsiness) may be intensified. At the same time, however, if TRAMADA is combined with a tranquillizer, for example, a favourable effect on pain sensation will probably be produced.
On concomitant or previous administration of carbamazepine, the analgesic effect and duration of action may be reduced.
Isolated convulsions have been observed on concomitant treatment with neuroleptics.

Benzodiazepines
Due to additive pharmacologic effect, the concomitant use of opioids with benzodiazepines increases the risk of respiratory depression, profound sedation, coma and death.

The concomitant use of opioids and benzodiazepines increases the risk of respiratory depression because of actions at different receptor sites in the central nervous system that control respiration. Opioids interact primarily at µ-receptors, and benzodiazepines interact at GABAA sites. When opioids and benzodiazepines are combined, the potential for benzodiazepines to significantly worsen opioid-related respiratory depression exists.

Reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate (see Precautions /Warnings).

Limit dosage and duration of concomitant use of benzodiazepines and opioids, and follow patients closely for respiratory depression and sedation.

Serotonergic Drugs
The concomitant use of opioids with other drugs that affect the serotonergic neurotransmitter system has resulted in serotonin syndrome. If concomitant use is warranted, carefully observe the patient, particularly during treatment initiation and dose adjustment. Discontinue if serotonin syndrome is suspected. Examples of serotonergic drugs are selective serotonin reuptake inhibitors (SSRIs), serotonin and norepinephrine reuptake inhibitors (SNRIs), tricyclic antidepressants (TCAs), triptans, 5-HT3 receptor antagonists, drugs that affect the serotonin neurotransmitter system (e.g. mirtazapine, trazodone, tramadol), monoamine oxidase (MAO) inhibitors (those intended to treat psychiatric disorders and also others, such as linezolid and intravenous methylene blue) (See Precautions/Warnings).

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION :

Unless otherwise prescribed, the following dosage guidelines apply:
Tramadol should not in any circumstances be used longer than therapeutically absolutely necessary.
If the nature and severity of the disease makes more prolonged analgesic treatment with tramadol necessary, a regular, careful and frequent check should be carried out (possibly by the use of treatment-free intervals) to determine whether and to what extent a medical requirement still exists.
Tramadol should be adjusted to the severity of pain and the patient's individual sensitivity. For moderately severe pain, tramadol should be given at the following dosage:

Adults and adolescents (12 years and older)
TRAMADA is not approved for use in patients below 12 years old.
1-2 capsules TRAMADA, equivalent to 50-100mg tramadol hydrochloride. Single oral dose of tramadol hydrochloride ranging from 50 - 200mg have provided relief of postoperative pain in patients who have undergo various types of surgery.
If a dose of 1 capsule of TRAMADA is not followed by adequate pain relief within 30-60 minutes, a single second dose of 1 capsule of TRAMADA may be given.
In severe pain, if clinical experience suggests that a higher analgesic dose is likely to be required, the initial dose may be 2 capsules of TRAMADA.
Not suitable for children weighing less than 25kg and cannot usually dosed individually for children under 14 years. An oral solution and an injectable solution are available for this purpose.
The duration of action of tramadol hydrochloride is an average of 4-8 hours, depending on the severity of pain, when therapeutic doses are given.
Daily doses of up to 8 capsules of TRAMADA (400mg tramadol hydrochloride) are generally adequate. In the treatment of tumour-induced pain, however, much higher daily doses may be used. (150 to 400mg daily in divided doses). The duration of action of tramadol may be prolonged in patients with disorders of renal or hepatic function. The need for dosage alterations in elderly patients is under discussion. There is some evidence that elderly patients require lower doses of tramadol than younger patients. Creatinine clearance should be monitored and used as a determining factor for dosage adjustment. Age alone is not sufficient criteria for dosage adjustment. While the usual oral tramadol hydrochloride dosage of 50-100mg every 4-6 hours as needed can be used in geriatric patients 65 years and older with normal renal and hepatic function, it is recommended that dosage not exceed 300mg daily in those than 75 years of age. Dosage of tramadol hydrochloride should be reduced in certain patients with renal or hepatic impairment by decreasing the frequency of administration. Adults and children 16 years of age and older with creatinine less than 30ml/min may receive an oral TRAMADA dosage of 50- 100 mg every 12 hours, not exceed 200mg daily. Since less than 10% dose tramadol hydrochloride is removed by haemodialysis, patients undergoing dialysis may receive their usual dosage on the day of dialysis. Adults and children 16 years of age and older with hepatic cirrhosis may receive a TRAMADA dosage of 50mg every 12 hours.
The capsules should be taken with a fluid, without relation to meal times.

Notes
The recommended dosages are guideline only. In principle, the lowest effective analgesic dose should be chosen. In the treatment of chronic pain, strict time-tabling of doses is preferred.

Paediatric population
The safety and efficacy of TRAMADA has not been studied in the paediatric population. Therefore, use of TRAMADA is not recommended in patients under 12 years of age.

DURATION OF TREATMENT
TRAMADA should not be given for a longer duration than absolutely required. If the nature and severity of the disease require long-term pain treatment with TRAMADA (intermittent treatment if necessary), careful investigation should be conducted at regular short intervals to justify the need for further treatment.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE, AND ANTIDOTE (S):

Symptoms : similar to those of centrally acting analgesics (opioids), particularly miosis, vomiting, cardiovascular collapse, reduced level of consciousness up to coma, convulsions and respiratory depression up to respiratory arrest.
Treatment : Keep the respiratory tract opened (aspiration) ; maintain respiratory and circulation depending on symptoms. An antidote for respiratory depression is NALOXONE (opiate antagonist).
Diazepam Injection should be given intravenously for convulsions since NALOXONE had no effect on convulsions according to animal studies.

PACKING / PACK SIZES :

Blister packed, with 10 capsules in a strip. The pack size is 10 strips in each paper box (100 capsules).

STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICAL PRECAUTIONS :

Store below 30 degrees Celsius.
Keep the medicine out of reach of children.
The shelf life is 4 years from date of manufacture.

NAME AND ADDRESS OF MANUFACTURER :

Charoen Bhaesaj Lab Co., Ltd.
32 Soi Phetkasem 21, Phetkasem Road, Pakklong Phasi Charoen, Phasi Charoen, Bangkok 10160, Thailand.

NAME AND ADDRESS OF DISTRIBUTOR IN MALAYSIA :

PROPHARM (M) SDN. BHD.
No.8, Jalan Udang Harimau 2, Medan Niaga Kepong, 51200 Kuala Lumpur, Malaysia.
Tel: 03-62436389 Fax: 03-62436385
email: propharm@gmail.com

APPLICATION 'S OVERVIEW OF THE PRODUCT :
Not Applicable.

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